

NetraJyoti AI

Bengali-first eye-care guidance before uncertainty becomes delay.

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The decision gap starts before a patient reaches care

For many Bengali-speaking rural users, the first challenge is deciding what to do next — not accessing a clinical diagnosis.

Uncertainty

“Is this serious? Can I wait? Where should I go?”

Trust gap

Family advice, informal care, and forwarded messages can delay formal eye care.

Access friction

Cost, travel, waiting time, communication barriers, and low digital confidence add friction.

Design opportunity: deliver a clear, understandable, safe next step before uncertainty turns into avoidable delay.

One MVP, four people it must work for

Rural Bengali-speaking user

Needs plain-language guidance without typing in English.

Caregiver / family member

Needs a shareable message and confidence to support timely care.

ASHA / community health worker

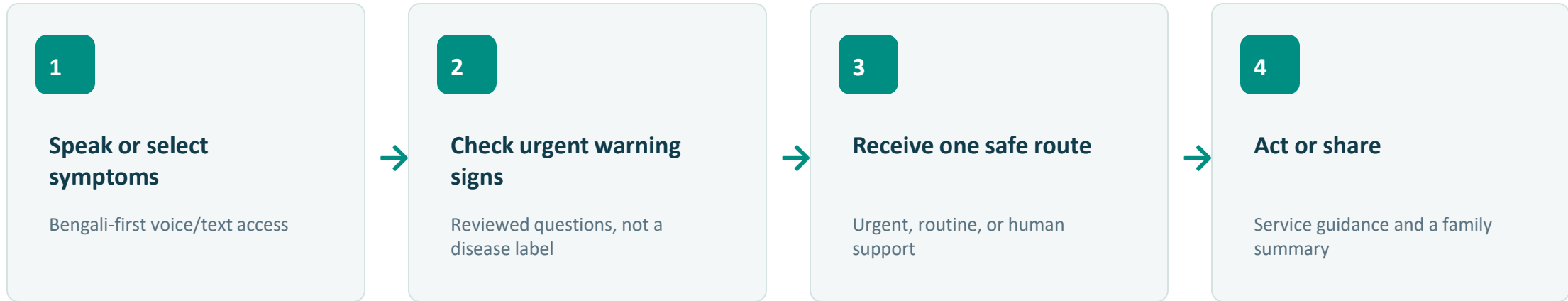
Needs an understandable route and nearby-support hand-off.

Outreach coordinator

Keeps eye-camp and service information current.

Primary principle: Bengali is primary; English is a smaller supportive layer for shared understanding.

NetraJyoti turns uncertainty into a safe next step



Safety boundary: NetraJyoti guides and navigates. It does not diagnose disease, prescribe treatment, or replace emergency care.

The reviewed route comes before any AI support

Only fixed, reviewed warning-sign answers determine the route. Free text, voice, location, patient details, and AI cannot change urgency.

1 Consent & start

2 Select symptoms

3 Urgent-sign check

4 Safe guidance

THREE POSSIBLE OUTCOMES

Urgent care

Routine guidance

Human support

The live demo covers all primary routes

Urgent care

Immediate care guidance; find care, share urgent message, continue to care.

Care navigation and family support

Routine guidance

Choose district/town, receive a time-bound eye-check recommendation.

Service direction and summary

Human support

Find ASHA/PHC/clinic or trusted-family support by area.

Local support options

The demonstration should show an urgent scenario, a non-urgent routine scenario, human-support fallback, and AI-disabled fallback.

AI is controlled, optional, and never decides the route

1. Deterministic rules

Reviewed Java safety-routing rules classify URGENT, ROUTINE, or HUMAN SUPPORT.

2. Optional AI summary

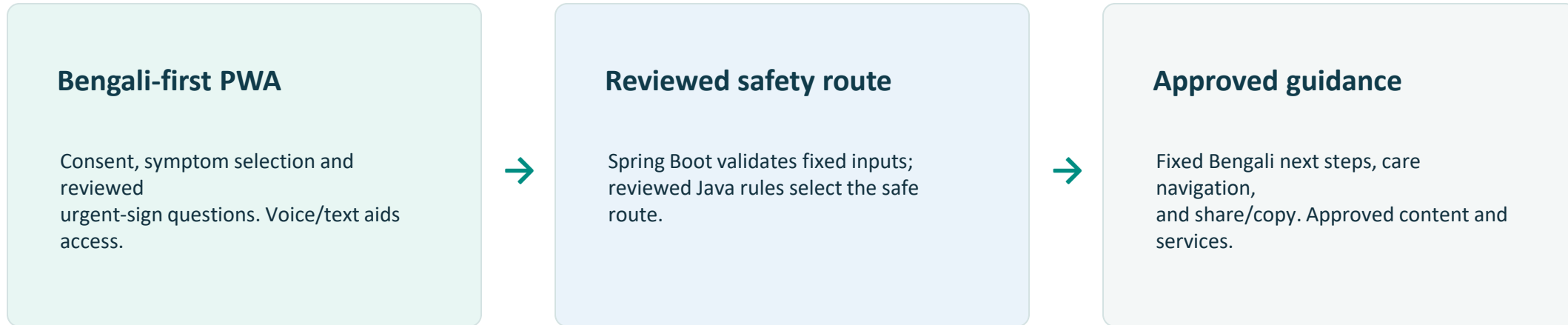
Feature flag: Enable AI Summary. It can draft a Bengali caregiver summary only after routing.

3. Safe fallback

If disabled, unavailable, or unsafe, show the approved Bengali fallback instead.

Guardrails: no diagnosis • no prescription • no autonomous care routing • no unvalidated health claim • no storage of unnecessary personal data

How the current MVP turns answers into safe guidance



Safety boundary: only reviewed identifiers select the route.

Free text, voice, location, patient details and AI cannot change urgency.

Delivery is traceable from Epic to working MVP

NET-1

NetraJyoti AI MVP Epic

Stories cover consent, symptoms, urgent routing, routine guidance, human support, service information, caregiver summary, and controlled AI.

51 story points

11

implementation stories

1

Epic: NET-1

3

core care outcomes

Kanban board and dashboard provide evidence of planned work, priority, workflow, and delivery progress.

JIRA EPIC • NET-1

What must be validated before a pilot

Safety and content

Test every reviewed safety rule; clinician/content review; service-directory verification.

Bengali usability

Observe comprehension, teach-back, consent, labels, voice fallback, and accessibility.

Technical resilience

Test APIs, route outcomes, permission denial, poor connectivity, share/copy fallback, privacy, and AI failure.

Real-world evidence

Five JTBD interviews, two-channel experiment, renewal interviews, and business-owner support interviews.

Pilot gate: proceed only when safety rules, usability findings, service-data checks, and privacy controls meet agreed acceptance criteria.

Launch through trusted local channels, not app discovery alone

01

Owned digital

Lightweight PWA, shareable guidance, SMS/WhatsApp-style family hand-off.

02

Partner-assisted

ASHA workers, PHCs, vision centres, eye camps, local outreach partners.

03

Service integrity

Verified directory, clearly labelled demo/confirmed services, update workflow for coordinators.

Launch promise: accessible Bengali guidance that helps people move from “What should I do?” to “Here is my safe next step.”

DEMO CLOSE

A safe next step in Bengali, before uncertainty becomes delay.

Demo checklist

- | | | |
|---|---------------|---|
| ✓ | Urgent path | Care navigation + family share |
| ✓ | Routine path | Service direction + caregiver summary |
| ✓ | Human support | ASHA / PHC / local support |
| ✓ | AI control | Enabled summary and safe Bengali fallback |

Thank you